2. SYNOPSIS

Name of Sponsor: Amgen Inc.

Name of Finished Product: Not applicable

Name of Active Ingredient*:* Bemarituzumab (AMG 552)

Title of Study: A Phase 1b/2 Study Evaluating the Safety, Tolerability, Efficacy, and Pharmacokinetics of Bemarituzumab in Combination With Other Anti-cancer Therapies in Subjects With Previously Untreated Advanced Gastric or Gastroesophageal Junction Cancer (FORTITUDE-103)

Investigator(s) and Study Center(s):

Phase 2 of this study was conducted at 39 study centers in Japan, South Korea, Singapore, and Taiwan (see Section 16.1.4).

Publication(s):

None as of the date of this report.

Study Period:

23 July 2024 (first participant enrolled)–07 November 2025 (first notification of early study termination submitted to a regulatory authority).

Study Phase: Phase 1b/2

Previous Reports for This Study:

Primary analysis clinical study report (CSR) (phase 1b), dated 19 December 2024 and CSR Addendum (phase 1b), dated 26 June 2025, and final analysis (FA) (phase 1b), dated 17 February 2026.

Study Rationale:

Gastric cancer (GC) represents the fifth most common cancer worldwide with over 1 million new cases globally each year (Bray F et al, 2018). Fibroblast growth factor receptor 2b (FGFR2b) is present on a substantial subset of gastric cancers and is a target for new therapies that may improve treatment outcomes. Though results vary significantly based on the assay used and the stage of the tumor, 20% to 30% of patients with GC have been reported to have FGFR2b-overexpressing tumors (Wainberg ZA et al, 2022; Catenacci DVT et al, 2021; Ishiwata T 2018; Ahn S et al, 2016). Fibroblast growth factor receptor signaling plays an important role in cell growth and survival (Katoh Y and Katoh M, 2006). Evidence of efficacy of adding

bemarituzumab to chemotherapy in participants with FGFR2b overexpressing GC is supported by results of phase 2 Study 20210113 (FPA144-004, FIGHT).

This study evaluated the safety and tolerability of bemarituzumab, an FGFR2b targeted therapy, in combination with anticancer therapies other than modified infusional 5-fluorouracil, leucovorin, and oxaliplatin, such as capecitabine and oxaliplatin (CAPOX), and CAPOX or S-1 and oxaliplatin (SOX) plus nivolumab (OPDIVO®) in phase 1b, and the efficacy of bemarituzumab in combination with SOX plus nivolumab in phase 2.

On 29 October 2025, Amgen decided to discontinue FORTITUDE-102 (Study 20210098), a phase 3 study evaluating bemarituzumab with chemotherapy and nivolumab in patients with FGFR2b+ advanced gastric or gastroesophageal junction cancer. After an ad-hoc data monitoring committee and Amgen review of the data, Amgen made the decision to prematurely discontinue the FORTITUDE-102 study due to lack of efficacy. FORTITUDE-103 (Study 20210099) is a similar study to FORTITUDE-102 (Study 20210098), with the primary distinction being the use of a different chemotherapy regimen (SOX vs folinic acid, 5-fluorouracil and oxaliplatin (FOLFOX)/CAPOX). Following Amgen's decision to prematurely discontinue FORTITUDE-102 (Study 20210098), Amgen made the decision further to a review of the entire program to also discontinue FORTITUDE-103 (Study 20210099) on 05 November 2025.

This report presents the results of the FA of phase 2 of FORTITUDE-103 (Study 20210099). The data presented are based on data cutoff date (21 November 2025) following early termination of the study. Participants have discontinued treatment with bemarituzumab and remain on standard-of-care treatment and continue to be followed for safety, including ocular toxicity.

Objectives, Endpoints: The protocol-specified objectives analyzed in the synoptic report include the following:

Phase 2/Part 2	
Objectives	**Endpoints**
Primary	
• To evaluate the efficacy of bemarituzumab in combination with S-1 and oxaliplatin (SOX) and nivolumab as assessed by objective response	• Objective response (defined as the best overall response of complete response [CR] or partial response [PR]) was determined by the investigator based on Response

	Evaluation Criteria in Solid Tumors Version 1.1 (RECIST v1.1). Confirmation of CR or PR was required at least 4 weeks after the initial detection of response
Secondary	
• To evaluate the safety and tolerability of bemarituzumab with SOX and nivolumab. • To evaluate the efficacy of bemarituzumab in combination with SOX and nivolumab as assessed by duration of response (DOR), time to response (TTR), disease control, progression-free survival (PFS), and overall survival (OS).	• Treatment-emergent adverse events, treatment-related adverse events, and clinically significant changes in vital signs, visual acuity, and clinical laboratory tests. • DOR was defined as the time from the first documentation of objective response to disease progression or death from any cause, whichever occurred first (as determined by the investigator based on RECIST v1.1). • TTR was defined as the time from the first dose of investigational product until the first documentation of objective response as determined by the investigator based on RECIST v1.1. • Disease control was defined as CR + PR + stable disease (SD) as determined by the investigator based on RECIST v1.1. • PFS was defined as the time from the first dose of investigational product until the first documentation of radiologic disease progression or death from any cause, whichever occurred first in the absence of subsequent anticancer therapy. PFS was censored at the last evaluable post-baseline disease assessment before subsequent anticancer therapy; otherwise, at first dose of investigational product. Progression was based on assessment by investigator per RECIST v1.1. • OS was defined as the time from the first dose of investigational product until death from any cause. Participants still alive were censored at the date last known to be alive.

The complete list of phase 1b primary and secondary objectives and endpoints is provided in Protocol Section 3 (Section 16.1.1 of this report).

Methodology:

This was a phase 1b/2, open-label, multicenter study to evaluate the safety, tolerability, pharmacokinetic (PK), and efficacy of bemarituzumab in combination with other anticancer therapies in participants with advanced gastric or gastroesophageal junction (GEJ) adenocarcinoma who had not received prior treatment for unresectable or metastatic disease.

This study consisted of 2 parts, a phase 1b dose-exploration and dose-expansion part (phase 1b) and a phase 2 single-arm part evaluating efficacy (part 2).

The design and methodology for phase 1b are described in the phase 1b FA CSR, dated 17 February 2026.

This report presents the design and methodology for phase 2 only.

Phase 2 was designed as a single-arm cohort to evaluate the efficacy of bemarituzumab in combination with SOX and nivolumab. Initiation of phase 2 occurred after enrollment in phase 1b cohort D had been completed, and the dose level review meeting (DLRM) confirmed the selected dose level, safety and tolerability profile of bemarituzumab in combination with SOX and nivolumab.

All enrolled participants received bemarituzumab in combination with SOX and nivolumab. Based on the DLRM recommendation following completion of phase 1b dose escalation (as of Protocol Amendment 4), phase 2 was conducted using bemarituzumab dose level 2 (22 mg/kg intravenously every 3 weeks, with an additional single dose of 11 mg/kg administered on day 8 of cycle 1), in combination with SOX and nivolumab.

Number of Participants Planned: 40 participants.

Diagnosis and Main Criteria for Eligibility:

Participants ≥ 18 years of age with histologically documented unresectable, locally advanced or metastatic gastric or GEJ adenocarcinoma (not amenable to curative therapy) (with FGFR2b at any FGFR2b 2+/3+ TC for cohorts A, C-e, and D-e, and FGFR2b at ≥ 10% 2+/3+ TC for phase 2). Participants had no contraindications to receive the combination therapy for a given cohort (eg, SOX with nivolumab), had evaluable disease (measurable or non-measurable) per RECIST v1.1 in phase 1b and

measurable disease per RECIST v1.1 in phase 2, and had not received prior treatment for unresectable or metastatic disease (except for a maximum of 1 dose of SOX with or without nivolumab allowed during pre-screening or screening for phase 2). Participants with known positive human epidermal growth factor receptor 2 status were excluded.

The complete list of inclusion and exclusion criteria is provided in Protocol Section 5.1 and 5.2, respectively (Section 16.1.1 of this report).

Investigational Products, Dose and Mode of Administration, Manufacturing Batch Number(s):

Information on investigational products, dose and mode of administration, and manufacturing batch numbers for phase 1b was provided in the phase 1b FA CSR dated 17 February 2026.

This report presents the investigational products, dose and mode of administration, and manufacturing batch numbers for phase 2 only.

Participants in phase 2 of the study were allowed to receive 1 cycle of SOX with or without nivolumab during pre-screening or screening if initiation of treatment was deemed urgent by the investigator. This 1 cycle of SOX with or without nivolumab was not required per protocol and was not considered part of the clinical study.

S-1 and oxaliplatin dosing was administered per protocol/local standard of care (SOC) as follows: S-1 was administered orally twice daily at 40 mg/m² per dose (ie, 40 mg/m² in the morning and 40 mg/m² in the evening), or per dose banding adjustment according to body surface area per local SOC, for 14 days followed by 7 days of rest (refer to Section 6.1.2 of the protocol). Oxaliplatin was administered at 130 mg/m² intravenously on day 1 of each 21-day cycle.

Nivolumab administration (21-day cycles):

- Administration commenced on day 1 of each cycle, after the end of bemarituzumab (cohorts C, C-e, D, D-e, and phase 2) infusion.

Day 1-nivolumab 360 mg intravenous over 30 minutes (± 10 minutes)

The manufacturing batch numbers are provided in Section 16.1.6 of this report.

Duration of Treatment:

Duration of treatment information for phase 1b was provided in the phase 1b FA CSR dated 17 February 2026.

This report presents the duration of treatment for phase 2 only.

The study consisted of a pre-screening period, a 28-day screening period, a treatment period, a safety follow-up (SFU) visit, and a long-term follow-up (LTFU) period.

In phase 2, the SFU visit was performed approximately 28 (+3) days after the end of all study treatment following permanent discontinuation for any reason, or 30 months after the first dose of bemarituzumab administered to the last enrolled participant, whichever occurred first.

In phase 2, for participants who permanently discontinued all study treatment for any reason, LTFU for survival commenced, and participants were followed by telephone approximately every 12 weeks (± 2 weeks) after the SFU visit for up to 30 months from the date the last enrolled participant received their first dose of bemarituzumab. If a participant was still receiving any study treatment at 30 months after the first dose of bemarituzumab administered to the last enrolled participant, all study treatment was ended, and the participant underwent an SFU visit 28 (+3) days later. The participant could continue to receive treatment under SOC; however, this was not considered study treatment.

Statistical Methods:

Sample size considerations: Assuming at least 40 participants received at least 1 dose of bemarituzumab, nivolumab, and SOX and had measurable disease per RECIST v1.1 (the Primary Analysis Set), the half-width of a 95% CI for the objective response rate (ORR) was expected to be no more than 15%.

Statistical method details for phase 1b were provided in phase 1b FA CSR dated 17 February 2026.

This report presents the statistical methods for phase 2 only.

For part 2, objective response, DOR, and TTR were summarized using the Primary Analysis Set, which included all participants who received at least 1 dose of bemarituzumab, nivolumab, and SOX and had measurable disease at baseline. Disease control, PFS, and OS were summarized using the Secondary Analysis Set, which

included all participants who received at least 1 dose of bemarituzumab, nivolumab, and SOX, regardless of measurable disease status at baseline (if the Primary and Secondary Analysis Sets differed, otherwise the Primary Analysis Set was used). Safety was summarized using the Safety Analysis Set, which included all participants who received at least 1 dose of bemarituzumab. Radiographic-based endpoints were based on investigator assessment of scans.

Objective response rate and disease control rate were summarized using frequency counts and percentages, together with 95% CIs calculated using the exact Clopper–Pearson method. Time-to-event endpoints, including DOR, PFS, and OS, were analyzed using the Kaplan–Meier method. Median event times, quartiles, and corresponding 95% CIs were estimated using the Brookmeyer and Crowley method. Kaplan–Meier estimates at landmark time points were also provided, with standard errors and corresponding 95% CIs calculated using Greenwood's formula. Time to response was summarized descriptively using summary statistics including median, minimum, maximum, mean, and standard deviation.

Summary of Results:

Participant Disposition

The FA was conducted following early termination of the study on 05 November 2025. Following study termination (Amgen's decision to terminate the study) and administrative close-out activities, treatment discontinuations were captured up to the data cutoff date (21 November 2025).

Study Discontinuation:

Overall, before the study termination 35 participants (81.4%) were still in the study, whereas 8 participants (18.6%) had discontinued the study due to death (Table 14-1.1.1).

Treatment Discontinuation at the Time of Study Termination:

As of the data cutoff date, 12 participants (27.9%) remained on bemarituzumab, whereas 31 participants (72.1%) had discontinued bemarituzumab. The reasons for discontinuation of bemarituzumab included disease progression (10 participants [23.3%]), adverse event (10 participants [23.3%]), decision by sponsor (8 participants [18.6%]), and subject request (3 participants [7.0%]) (Table 14-1.1.1).

Baseline Demographics: (Table 14-2.1.1**)**

Sex: 32 men (74.4%), 11 women (25.6%).

Age: median 72.0 (range: 37 to 84) years.

Race: Asian 43 participants (100.0%).

Ethnicity: not Hispanic or Latino 43 participants (100.0%).

Efficacy Results:

Primary Endpoint:

Objective Response Rate:

The ORR was 48.8% (95% CI: 33.3, 64.5). Partial responses were observed in 21 participants, and no participants achieved a CR (Table 14-4.1.1).

Secondary Endpoints:

Duration of Response:

The median DOR was 6.9 months (95% CI: 4.2, not estimable [NE]) for the 21 participants achieving a response. Kaplan-Meier (KM) estimated DOR rates at 3, 6, and 9 months were 100.0%, 51.9%, and 39.0%, respectively (Table 14-4.1.1).

Time to Response:

The median TTR was 1.38 (range: 1.2 to 4.1) months (Table 14-4.1.1).

Disease Control:

Of the 43 participants, 39 participants achieved disease control, resulting in a disease control rate of 90.7% (95% CI: 77.9, 97.4) (Table 14-4.1.1).

Progression-free Survival:

Of the 43 participants, 18 participants (41.9%) had a PFS event (15 participants [34.9%] disease progression; 3 participants [7.0%] died), and 25 participants (58.1%) were censored. Two deaths (1 due to pneumonia and 1 due to suicide) occurred outside the treatment-emergent adverse event window. A third participant died due to disease progression; however, progression was not confirmed by radiographic assessment, and, therefore, this death was not classified as a progression event but was included as a death event in the PFS analysis. The median PFS was 7.1 months (95% CI: 5.3, 8.3).

Kaplan-Meier estimated PFS rates at 3, 6, 9, and 12 months were 83.1%, 55.6%, 26.0%, and 17.4%, respectively (Table 14-4.1.3 and Figure 14-4.1.2).

Overall Survival:

Of the 43 participants, 9 participants (20.9%) had died, and 34 participants (79.1%) were censored. Although only 8 participants were reported as deceased in the Participant Disposition section, 1 additional participant was identified as deceased based on alternative sources used for the OS analysis; this participant did not complete the end-of-study page. The median OS was 14.1 months (95% CI: 10.0, NE). Kaplan-Meier-estimated OS rates at 3, 6, 9, and 12 months were 95.1%, 87.0%, 87.0%, and 59.6%, respectively (Table 14-4.1.2 and Figure 14-4.1.1).

Other Results:

Pharmacokinetic Results:

Due to early study termination and sparse PK sampling, only descriptive summaries of serum concentrations are reported. A PK data summary was performed using decoupled clinical and PK data extracts dated 21 November 2025 and 12 March 2026, respectively. Pharmacokinetic records with sample collection dates after the clinical data extraction date were not included in the analysis. Noncompartmental analysis was not performed. Listings of all serum PK samples collected along with descriptive statistics are presented in Section 16.1.13.2 of this report.

Anti-bemarituzumab Antibody Results

At any time point, 9 of 43 participants (20.9%) who received bemarituzumab tested positive for antidrug antibody (ADA) (ADA prevalence); however, most ADA-positive participants were positive at baseline and did not experience a treatment-boosted response. Of the participants with a post-baseline antibody result, 1 of 42 participants (2.4%) tested positive for a treatment-emergent ADA (ADA incidence). No participants tested positive for anti-bemarituzumab neutralizing antibodies (Table 14-10.1.1).

Safety Results:

Extent of Exposure:

All 43 participants received at least 1 dose of bemarituzumab were included in the Safety Analysis Set. The median duration of bemarituzumab treatment was 12.14 weeks (range: 1.1 to 65.1). The median number of bemarituzumab cycles received was 4.0 (range: 1 to 9) (Table 14-5.1.1).

A summary of bemarituzumab dose modification is provided in Table 14-5.2.1.

Overall Summary of Treatment-emergent Adverse Events:

All 43 participants (100.0%) experienced at least 1 treatment-emergent adverse event (herein referred to as adverse events) (Table 14-6.1.1).

Most Frequently Reported Treatment-emergent Adverse Events

The most frequently reported adverse events (occurring in ≥ 20% of participants) by preferred term were visual acuity reduced (33 participants [76.7%]), punctate keratitis (26 participants [60.5%]), peripheral sensory neuropathy (21 participants [48.8%]), decreased appetite (20 participants [46.5%]), nausea (19 participants [44.2%]), neutrophil count decreased (15 participants [34.9%]), diarrhoea (14 participants [32.6%]), malaise (13 participants [30.2%]), dysgeusia and platelet count decreased (each, 11 participants [25.6%]), anaemia and stomatitis (each, 10 participants [23.3%]) and constipation (9 participants [20.9%]) (Table 14-6.2.1).

Treatment-related Adverse Events:

Treatment-related adverse events were reported in 29 participants (67.4%). The most frequently reported treatment-related adverse events (occurring in ≥ 10% of participants) by preferred term were punctate keratitis and visual acuity reduced (each, 24 participants [55.8%]), corneal epithelium defect and limbal stem cell deficiency (each, 7 participants [16.3%]), nausea and infusion related reaction (each, 5 participants [11.6%]) (Table 14-6.4.1).

Grade ≥ 3 Adverse Events:

Grade ≥ 3 adverse events were reported in 35 participants (81.4%). The most frequently reported grade ≥ 3 adverse events (occurring in ≥ 5% of participants) by preferred term were visual acuity reduced (17 participants [39.5%]), punctate keratitis (13 participants [30.2%]), limbal stem cell deficiency (7 participants [16.3%]), corneal epithelium defect (5 participants [11.6%]), anaemia and diarrhoea (each, 4 participants [9.3%]), and ulcerative keratitis and neutropenia (each, 3 participants [7.0%]) (Table 14-6.3.3).

Interruptions of Investigational Product due to Adverse Events:

Adverse events leading to interruption of the investigational product were reported in 24 participants (55.8%). The most frequently reported adverse events leading to interruption of the investigational product (occurring in ≥ 5% of participants) by preferred term were visual acuity reduced (11 participants [25.6%]), punctate keratitis

(9 participants [20.9%]), limbal stem cell deficiency (6 participants [14.0%]), and corneal epithelium defect (4 participants [9.3%]) (Table 14-6.3.4).

Withdrawals from Investigational Product due to Adverse Events:

Adverse events leading to discontinuation of the investigational product were reported in 10 participants (23.3%). The preferred terms were visual acuity reduced (9 participants [20.9%]) and punctate keratitis (3 participants [7.0%]) (Table 14-6.3.5). All adverse events leading to discontinuation of the investigational product were considered treatment-related (Table 14-6.4.1).

Serious Adverse Events:

Serious adverse events were reported in 18 participants (41.9%). The most frequently reported serious adverse events by preferred term were diarrhoea (3 participants [7.0%]), followed by hypokalaemia (2 participants [4.7%]); all other events occurred in 1 participant each (2.3%), including anaemia, thrombocytopenia, colitis ischaemic, enteritis, gastric stenosis, immune-mediated enterocolitis, obstruction gastric, pancreatitis acute, stomatitis, asthenia, pyrexia, bile duct stenosis, drug-induced liver injury, immune-mediated hepatitis, anaphylactic reaction, device-related infection, infection, infusion related reaction, decreased appetite, hyperglycaemic hyperosmolar nonketotic syndrome, pneumonitis, and peripheral artery thrombosis (Table 14-6.3.2).

Treatment-related serious adverse events were reported in 3 participants (7.0%). No treatment-related serious adverse events by preferred term were reported in more than 1 participant. Reported events included thrombocytopenia, diarrhoea, stomatitis, infusion related reaction, and hypokalaemia (1 participant, each [2.3%]) (Table 14-6.4.2).

Participant narratives are provided in Section 14.6.2 of this report.

Death:

No treatment-emergent fatal adverse events occurred during phase 2 (Table 14-6.3.6).

Other Evaluations Related to Safety:

Changes in visual acuity (logarithm of the minimum angle of resolution [LogMAR]) were observed during the study and are provided in Table 14-6.5.1 and Table 14-6.5.2.

The distribution of worst on-study visual acuity grades is presented in Table 14-6.5.3. In unilateral (in better eye) assessments, 39.5% of participants had no worsening from baseline (grade 0). Worsening of > 0 to ≤ 2 lines (grade 1), 3–4 lines (grade 2), 5–6 lines (grade 3), and > 6 lines (grade 4) occurred in 14.0% (6 participants), 18.6%

(8 participants), 14.0% (6 participants), and 9.3% (4 participants), respectively. Among the 6 participants (14.0%) with grade 3 visual acuity reduced, 2 participants improved to baseline and 4 participants improved to grade ≤ 2, and 2 remained in follow-up at the time of data cutoff. Similarly, among the 4 participants (9.3%) with grade 4 visual acuity reduction, 1 participant improved to grade 1, and 3 participants remained in follow-up at the time of data cutoff. Subsequent ophthalmology examinations performed after data cutoff demonstrated improvement in visual acuity to grade ≤ 2 in these 3 participants.

Clinical Laboratory Evaluation:

Laboratory values are provided in Table 14-7.1.1 to Table 14-7.1.17. Shifts from baseline in grade using Common Terminology Criteria for Adverse Events criteria are provided in Table 14-7.2.1 to Table 14-7.2.18.

No participants had laboratory values that met the Hy's Law criteria (Table 14-7.3.1).

Other Safety Evaluations:

No clinically meaningful changes from baseline to post-baseline time points were observed for vital signs (systolic blood pressure, diastolic blood pressure, respiratory rate, heart rate, and body temperature) during the study (Table 14-8.1.1).

Physical Findings:

No meaningful changes from baseline to post-baseline time points were observed in physical findings (weight) (Table 14-8.1.2).

Conclusions:

- Following Amgen's decision to prematurely discontinue FORTITUDE-102, Amgen made the decision to also discontinue FORTITUDE-103 (Study 20210099). Due to early discontinuation of the study, interpretation of the efficacy results is limited, and definitive conclusions regarding the regimen's efficacy cannot be established.
- The safety profile of bemarituzumab in combination with SOX and nivolumab was generally consistent with the known safety profile of bemarituzumab and the individual components of the regimen, while acknowledging the limited exposure resulting from early termination of the study.
- The most frequently reported adverse events were ocular and gastrointestinal in nature.

- Interpretation of benefit-risk is limited because of early study discontinuation and the absence of a comparator arm.

Report Date:

Document Version	Date
Primary Analysis (phase 1b)	19 December 2024
Addendum (phase 1b)	26 June 2025
Final Analysis (phase 1b)	17 February 2026
Final Analysis (phase 2)	19 June 2026